Department of Health Care Finance

www.dhcf.dc.gov
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Table HT0-1

Description	FY 2024 Actual	FY 2025 Actual	FY 2026 Approved	FY 2027 Proposed	% Change from FY 2026
OPERATING BUDGET	$4,401,503,659	$4,411,633,691	$5,504,769,104	$5,335,741,431	-3.1
FTEs	331.7	342.7	390.9	395.2	1.1
CAPITAL BUDGET	$147,131,622	$145,236,347	$0	$2,535,300	N/A
FTEs	0.0	0.0	0.0	0.0	N/A

The mission of the Department of Health Care Finance (DHCF) is to improve health outcomes by providing access to comprehensive, cost-effective, and quality health care services for residents of the District of Columbia.

Summary of Services

The Department of Health Care Finance provides health care services to low-income children, adults, the elderly, and persons with disabilities. Nearly 290,000 District of Columbia residents (more than 40 percent of all residents) receive health care services through DHCF's Medicaid and Alliance programs. DHCF strives to provide these services in the most appropriate and cost-effective settings possible.

The agency's FY 2027 proposed budget is presented in the following tables:

FY 2027 Proposed Gross Funds Operating Budget and FTEs, by Revenue Type

Table HT0-2 contains the proposed FY 2027 budget and Full-Time Equivalents by revenue type compared to the FY 2026 approved budget. It also provides FY 2024 and FY 2025 actual data.

Table HT0-2
($ in thousands)

	Dollars in Thousands						Full-Time Equivalents					
Appropriated Fund	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	% Change*	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	% Change
GENERAL FUND												
LOCAL FUND	1,049,998	1,051,847	1,054,707	1,118,310	63,603	6.0	146.6	145.3	176.0	171.2	-4.7	-2.7
DEDICATED TAXES	114,647	235,337	285,940	236,167	-49,774	-17.4	7.6	6.6	6.6	6.6	0.0	0.0
SPECIAL PURPOSE REVENUE FUNDS	3,928	3,446	5,492	4,781	-711	-13.0	14.0	13.8	15.1	15.1	0.0	0.0
TOTAL FOR GENERAL FUND	**1,168,573**	**1,290,630**	**1,346,139**	**1,359,257**	**13,118**	**1.0**	**168.3**	**165.8**	**197.6**	**192.9**	**-4.7**	**-2.4**

Table HT0-2
($ in thousands)

Appropriated Fund	Dollars in Thousands						Full-Time Equivalents					
	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	% Change*	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	% Change
FEDERAL RESOURCES												
FEDERAL GRANT FUND - FPR	2,218	3,245	10,400	10,238	-162	-1.6	0.0	10.0	8.0	13.0	5.0	62.5
FEDERAL MEDICAID PAYMENTS	3,230,713	3,117,759	4,148,230	3,966,246	-181,984	-4.4	163.5	167.0	185.3	189.3	4.1	2.2
TOTAL FOR FEDERAL RESOURCES	**3,232,931**	**3,121,004**	**4,158,630**	**3,976,485**	**-182,146**	**-4.4**	**163.5**	**177.0**	**193.3**	**202.3**	**9.1**	**4.7**
GROSS FUNDS	**4,401,504**	**4,411,634**	**5,504,769**	**5,335,741**	**-169,028**	**-3.1**	**331.7**	**342.7**	**390.9**	**395.2**	**4.3**	**1.1**

* Percent change is based on whole dollars.

Note: If applicable, for a breakdown of each Grant (Federal and Private) and Special Purpose Revenue type, please refer to **Schedule 80 Agency Summary by Revenue Source** in the **FY 2027 Operating Appendices** located on the Office of the Chief Financial Officer's website.

FY 2027 Proposed Operating Budget, by Account Group

Table HT0-3 contains the proposed FY 2027 budget at the Account Group level compared to the FY 2026 approved budget. It also provides FY 2024 and FY 2025 actual expenditures.

Table HT0-3
($ in thousands)

Account Group	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	% Change*
701100C-CONTINUING FULL TIME	34,338	35,472	41,181	42,681	1,500	3.6
701200C-CONTINUING FULL TIME - OTHERS	1,658	2,279	2,550	2,088	-462	-18.1
701300C-ADDITIONAL GROSS PAY	323	97	2	0	-2	-100.0
701400C-FRINGE BENEFITS - CURR PERSONNEL	8,118	8,894	10,101	10,574	472	4.7
701500C-OVERTIME PAY	124	67	0	0	0	N/A
SUBTOTAL PERSONNEL SERVICES (PS)	**44,560**	**46,810**	**53,834**	**55,343**	**1,509**	**2.8**
711100C-SUPPLIES & MATERIALS	110	58	134	121	-14	-10.1
712100C-ENERGY COMM & BLDG RENTALS	2,302	2,364	2,638	2,681	43	1.6
713100C-OTHER SERVICES & CHARGES	3,194	3,002	4,200	1,732	-2,468	-58.8
713101C-SECURITY SERVICES	43	79	140	238	98	69.7
713200C-CONTRACTUAL SERVICES - OTHER	145,909	145,756	192,083	223,495	31,412	16.4
714100C-GOVERNMENT SUBSIDIES & GRANTS	4,195,643	4,200,764	5,238,838	5,036,956	-201,881	-3.9
717100C-PURCHASES EQUIPMENT & MACHINERY	9,741	12,801	12,895	15,175	2,280	17.7
717200C-RENTALS EQUIPMENT & OTHER	0	0	7	0	-7	-100.0
SUBTOTAL NONPERSONNEL SERVICES (NPS)	**4,356,943**	**4,364,824**	**5,450,935**	**5,280,398**	**-170,537**	**-3.1**
GROSS FUNDS	**4,401,504**	**4,411,634**	**5,504,769**	**5,335,741**	**-169,028**	**-3.1**

* Percent change is based on whole dollars.

FY 2027 Proposed Operating Budget and FTEs, by Division/Program and Activity

Table HT0-4 contains the proposed FY 2027 budget by division/program and activity compared to the FY 2026 approved budget. It also provides FY 2024 and FY 2025 actual data. For a more comprehensive explanation of divisions/programs and activities, please see the Division/Program Description section, which follows the table.

Table HT0-4
($ in thousands)

	Dollars in Thousands					Full-Time Equivalents				
Division/Program and Activity	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026
AFO002-AGENCY ACCOUNTING SERVICES	3,927	4,885	6,463	7,131	668	8.1	8.9	11.0	11.0	0.0
AFO003-AGENCY BUDGETING AND FINANCIAL MANAGEMENT SERVICES	751	863	889	974	85	5.4	5.3	7.0	7.0	0.0
AFO005-AGENCY /CLUSTER FINANCIAL EXECUTIVE ADMINISTRATION	354	356	374	387	13	1.8	1.8	2.0	2.0	0.0
AFO011-P-CARD CLEARING	5	0	0	0	N/A	0.0	0.0	0.0	0.0	N/A
AFO000-AGENCY FINANCIAL OPERATIONS	**5,036**	**6,104**	**7,726**	**8,491**	**765**	**15.3**	**16.0**	**20.0**	**20.0**	**0.0**
AMP003-COMMUNICATIONS	817	0	36	36	0	4.5	0.0	0.0	0.0	N/A
AMP005-CONTRACTING AND PROCUREMENT	1,576	1,767	1,869	2,115	246	11.7	12.4	14.0	14.0	0.0
AMP006-CUSTOMER EXPERIENCE	2,846	0	0	0	N/A	20.9	0.0	0.0	0.0	N/A
AMP010-GRANTS ADMINISTRATION	1,692	0	0	0	N/A	3.7	0.0	0.0	0.0	N/A
AMP011-HUMAN RESOURCE SERVICES	982	875	1,244	1,033	-211	9.0	7.1	8.0	8.0	0.0
AMP012-INFORMATION TECHNOLOGY SERVICES	5,620	9,894	13,787	17,476	3,689	7.0	15.9	17.0	16.2	-0.8
AMP014-LEGAL SERVICES	1,221	1,158	1,480	1,900	419	8.2	7.9	8.9	8.9	0.0
AMP019-PROPERTY, ASSET, AND LOGISTICS MANAGEMENT	6,083	3,210	3,793	3,969	176	14.5	5.3	7.0	7.0	0.0
AMP021-RATES, REIMBURSEMENT, FINANCIAL ANALYSIS	4,225	-55	0	0	N/A	16.3	0.0	0.0	0.0	N/A
AMP026-TRAINING AND DEVELOPMENT	0	97	0	0	N/A	0.0	0.0	0.0	0.0	N/A
AMP030-EXECUTIVE ADMINISTRATION	972	941	1,691	1,593	-98	6.3	8.0	10.0	11.0	1.0
AMP037-SENIOR DEPUTY DIRECTOR/MEDICAID DIRECTOR	0	2,538	14,146	26,926	12,780	0.0	12.5	15.0	15.0	0.0
AMP038-SENIOR DEPUTY DIRECTOR/FINANCE	0	4,405	6,602	5,212	-1,391	0.0	17.9	23.0	22.0	-1.0
AMP039-CHIEF OPERATING OFFICE	0	1,661	1,468	2,095	627	0.0	7.1	7.0	7.0	0.0
AMP040-DATA ANALYTICS AND RESEARCH ADMINISTRATION	0	1,318	2,523	2,388	-135	0.0	9.7	13.0	13.0	0.0
AMP041-PROGRAM INTEGRITY	0	4,211	5,565	5,583	18	0.0	29.3	36.0	35.0	-1.0
AMP042-HEALTH CARE OMBUDSMAN	0	3,448	5,342	5,284	-59	0.0	21.9	26.0	26.0	0.0
AMP000-AGENCY MANAGEMENT PROGRAM	**26,033**	**35,467**	**59,546**	**75,608**	**16,062**	**102.2**	**155.0**	**184.9**	**183.1**	**-1.7**
AMP043-HEALTH CARE DELIVERY MGT SUPPORT SVCS	0	1,221	1,509	1,548	39	0.0	8.0	10.0	9.0	-1.0
AMP044-MANAGED CARE MGT	0	8,566	9,425	11,024	1,598	0.0	6.2	6.0	7.0	1.0

Table HT0-4
($ in thousands)

Division/Program and Activity	Dollars in Thousands					Full-Time Equivalents				
	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026
AMP045-CHILDREN'S HEALTH SERVICES	0	857	1,237	1,655	418	0.0	3.5	4.0	7.6	3.6
AMP046-HEALTH CARE QUALITY AND HEALTH OUTCOMES	0	2,247	2,635	3,461	826	0.0	6.2	7.0	7.0	0.0
AMP047-CLINICIANS, RX AND ACUTE CARE	0	7,841	0	0	N/A	0.0	0.0	0.0	0.0	N/A
AMP066-HEALTH CARE DELIVERY MANAGEMENT	**0**	**20,732**	**14,806**	**17,688**	**2,882**	**0.0**	**23.9**	**27.0**	**30.6**	**3.6**
AMP048-LONG TERM CARE SUPPORT SERVICES	0	410	809	675	-134	0.0	5.3	6.0	6.0	0.0
AMP049-OVERSIGHT	0	2,212	2,271	2,539	268	0.0	20.7	17.0	18.0	1.0
AMP050-OPERATIONS	0	7,244	18,654	13,787	-4,867	0.0	14.6	16.0	16.0	0.0
AMP051-INTAKE AND ASSESSMENT	0	8,647	8,442	8,396	-46	0.0	5.8	7.0	6.0	-1.0
AMP067-LONG TERM CARE PROGRAM	**0**	**18,513**	**30,176**	**25,398**	**-4,778**	**0.0**	**46.5**	**46.0**	**46.0**	**0.0**
AMP052-HEALTH CARE POLICY	0	740	1,021	892	-130	0.0	6.2	7.0	7.0	0.0
AMP053-HEALTH CARE POLICY AND RESEARCH SUPPORT SERVICES	0	702	1,197	1,430	233	0.0	2.7	4.0	5.0	1.0
AMP054-ELIGIBILITY POLICY & OVERSIGHT	0	3,672	4,402	4,650	248	0.0	25.7	29.0	30.0	1.0
AMP068-HEALTH CARE POLICY AND RESEARCH	**0**	**5,113**	**6,621**	**6,972**	**351**	**0.0**	**34.5**	**40.0**	**42.0**	**2.0**
AMP055-DCAS PROGRAM MANAGEMENT	0	1,621	1,828	6,275	4,447	0.0	3.9	4.3	4.6	0.3
AMP056-DCAS PROJECT MANAGEMENT	0	2,903	3,923	5,634	1,711	0.0	6.2	5.2	5.6	0.4
AMP057-DCAS HHS FUNCTIONAL	0	856	851	849	-3	0.0	5.4	6.1	6.5	0.4
AMP058-DCAS ORGANIZATIONAL CHANGE MANAGEMENT	0	9,136	12,422	14,521	2,099	0.0	6.2	6.9	7.4	0.5
AMP059-DCAS INFORMATION TECHNOLOGY	0	54,434	53,078	63,022	9,944	0.0	6.2	6.9	7.4	0.5
AMP069-DC ACCESS SYSTEM	**0**	**68,949**	**72,102**	**90,300**	**18,198**	**0.0**	**27.7**	**29.4**	**31.5**	**2.0**
AMP060-CLAIMS MANAGEMENT	0	48,804	64,208	61,410	-2,798	0.0	12.6	14.0	14.0	0.0
AMP061-HCOA SUPPORT SERVICES	0	461	483	494	11	0.0	2.7	3.0	3.0	0.0
AMP062-PUBLIC AND PRIVATE PROVIDER SERVICES	0	3,217	6,425	6,417	-8	0.0	8.0	8.0	8.0	0.0
AMP070-HEALTH CARE OPERATIONS	**0**	**52,482**	**71,116**	**68,321**	**-2,795**	**0.0**	**23.2**	**25.0**	**25.0**	**0.0**
AMP063-HC REFORM AND INNOVATIVE SUPPORT SVS	0	380	611	681	70	0.0	2.7	4.0	4.0	0.0
AMP064-GRANTS ADMINISTRATION FUNCTION	0	3,527	10,208	15,278	5,071	0.0	2.8	4.0	6.0	2.0
AMP065-HIE: HEALTH INFORMATION EXCHANGE	0	7,743	11,221	11,403	182	0.0	6.5	7.0	7.0	0.0
AMP071-HEALTH CARE REFORM AND INNOVATION	**0**	**11,650**	**22,040**	**27,362**	**5,323**	**0.0**	**12.0**	**15.0**	**17.0**	**2.0**
H02301-E & E SYSTEMS	46,339	111	0	0	N/A	15.6	0.9	0.0	0.0	N/A
H02302-E&E OVERSIGHT & MANAGEMENT	812	0	0	0	N/A	9.0	0.0	0.0	0.0	N/A
H02305-E&E SUPPORT - PMO/SME - DCAS	21,832	24	0	0	N/A	15.1	0.0	0.0	0.0	N/A

Table HT0-4
($ in thousands)

Division/Program and Activity	Dollars in Thousands					Full-Time Equivalents				
	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026	Actual FY 2024	Actual FY 2025	Approved FY 2026	Proposed FY 2027	Change from FY 2026
HS0023-ELIGIBILITY & ENROLLMENT	**68,983**	**135**	**0**	**0**	**0**	**39.7**	**0.9**	**0.0**	**0.0**	**0.0**
H02401-HEALTH CARE AGENCY MONITORING AND KPIS	1,594	-1	0	0	N/A	8.1	0.0	0.0	0.0	N/A
H02402-HEALTH CARE DATA ANALYTICS	1,369	0	0	0	N/A	8.1	0.0	0.0	0.0	N/A
H02403-HEALTH CARE INNOVATION	7,682	-67	0	0	N/A	7.4	0.0	0.0	0.0	N/A
HS0024-HEALTH CARE ANALYTICS	**10,646**	**-68**	**0**	**0**	**0**	**23.6**	**0.0**	**0.0**	**0.0**	**0.0**
H02501-CLAIMS PROCESSING & QUALITY ASSURANCE/CONTROL	47,021	-4	0	0	N/A	15.4	0.0	0.0	0.0	N/A
HS0025-MMIS SYSTEM AND INFRASTRUCTURE	**47,021**	**-4**	**0**	**0**	**0**	**15.4**	**0.0**	**0.0**	**0.0**	**0.0**
H02601-ASSESSMENTS AND CARE COORDINATION	1,009	0	0	0	N/A	7.2	0.0	0.0	0.0	N/A
H02602-FRAUD, WASTE, AND ABUSE	4,155	-8	0	0	N/A	25.3	0.0	0.0	0.0	N/A
H02603-POLICY	4,757	-152	0	0	N/A	28.0	0.0	0.0	0.0	N/A
H02604-PROVIDER OVERSIGHT	40,647	252	0	0	N/A	56.9	0.0	0.0	0.0	N/A
H02605-QUALITY & HEALTH OUTCOMES	3,292	324	395	0	-395	11.8	3.0	3.6	0.0	-3.6
HS0026-PROGRAM OVERSIGHT	**53,859**	**416**	**395**	**0**	**-395**	**129.2**	**3.0**	**3.6**	**0.0**	**-3.6**
H02703-1115/1915 - MEDICAID	413,218	431,431	498,998	619,847	120,849	0.0	0.0	0.0	0.0	N/A
H02704-FFS - CHILDLESS ADULTS (GROUP 8)	9,733	9,723	69,896	14,511	-55,385	0.0	0.0	0.0	0.0	N/A
H02705-FFS - CHIP	8,368	8,046	13,375	10,938	-2,437	0.0	0.0	0.0	0.0	N/A
H02706-FFS - MEDICAID	1,449,063	1,398,887	1,433,980	1,551,590	117,610	1.7	0.0	0.0	0.0	N/A
H02707-MCO - ALLIANCE	147,448	162,023	0	260	260	0.0	0.0	0.0	0.0	N/A
H02708-MCO - CHILDLESS ADULTS (GROUP 8)	712,779	715,257	1,129,395	908,288	-221,106	0.0	0.0	0.0	0.0	N/A
H02709-MCO - CHIP	54,132	62,684	97,779	91,950	-5,829	0.0	0.0	0.0	0.0	N/A
H02710-MCO - IMMIGRANT CHILDREN	19,114	22,362	0	0	N/A	0.0	0.0	0.0	0.0	N/A
H02711-MCO - MEDICAID	1,130,795	1,124,256	1,658,576	1,408,848	-249,728	0.0	0.0	0.0	0.0	N/A
H02712-MCO- WAIVER	151,402	192,237	196,846	317,450	120,604	0.0	0.0	0.0	0.0	N/A
H02713-HCBS ARPA INITIATIVE	78,804	79,792	0	0	N/A	4.6	0.0	0.0	0.0	N/A
H02714-INDIGENT CARE	0	0	121,396	91,918	-29,478	0.0	0.0	0.0	0.0	N/A
HS0027-PROVIDER SERVICES	**4,174,857**	**4,206,697**	**5,220,241**	**5,015,601**	**-204,640**	**6.3**	**0.0**	**0.0**	**0.0**	**0.0**
PRG001-NO PROGRAM	15,069	-14,553	0	0	N/A	0.0	0.0	0.0	0.0	N/A
PRG000-NO PROGRAM	**15,069**	**-14,553**	**0**	**0**	**0**	**0.0**	**0.0**	**0.0**	**0.0**	**0.0**
TOTAL PROPOSED OPERATING BUDGET	**4,401,504**	**4,411,634**	**5,504,769**	**5,335,741**	**-169,028**	**331.7**	**342.7**	**390.9**	**395.2**	**4.3**

(Change is calculated by whole numbers and numbers may not add up due to rounding)

Note: For more detailed information regarding the proposed funding for the activities within this agency's programs, please see **Schedule 30-PBB Program Summary by Activity.** For detailed information on this agency's Cost Center structure as reflected in the District's Chart of Accounts, please see **Schedule 30-CC FY 2027 Proposed Operating Budget and FTEs, by Division/Office.** Both schedules can be found in the **FY 2027 Operating Appendices**, located on the Office of the Chief Financial Officer's website. Additional information on this agency's interagency agreements can be found in **Appendix H** in the **Executive Summary, Volume 1**.

Division Description

The Department of Health Care Finance operates through the following 9 divisions:

Health Care Delivery Management – ensures that quality services and practices pervade all activities that affect the delivery of health care to beneficiaries served by the District's Medicaid, Children's Health Insurance Program (CHIP), and Alliance programs. HCDM accomplishes this through informed benefit design; use of prospective, concurrent and retrospective utilization management; ongoing program evaluation; and the application of continuous quality measurement and improvement practices in furnishing preventive, acute, and chronic/long-term care services to children and adults through DHCF's managed care contractors and institutional and ambulatory fee-for-service providers.

This division contains the following 4 activities**:**

- **Health Care Delivery Management Support Services** – provides administrative support functions to the Health Care Delivery Management division;
- **Managed Care Management** – provides oversight, evaluation, and enforcement of contracts with organizations managing the care and service delivery of Medicaid and Alliance beneficiaries, along with providing oversight and enrollment of eligible beneficiaries;
- **Children's Health Services** – develops, implements, and monitors policies, benefits and practices for children's health care services, including HealthCheck/EPSDT, CHIP, and the Immigrant Children's Program; and
- **Health Care Quality and Health Outcomes** – continuously improves the quality (safe, effective, patient-centered, timely, efficient, and equitable services) of health care delivered by programs administered by DHCF; and ensures that quality and performance improvement principles and practices pervade all the components and activities that impact the delivery and outcomes of health care services to patients served by the District's Medicaid, CHIP, and Alliance programs.

Long-Term Care Program – provides operations support, oversight and monitoring of programs targeted to the elderly, persons with physical disabilities, persons with intellectual and developmental disabilities and individuals who are dually eligible for both Medicare and Medicaid. Through program development, day-to-day operations, and oversight and monitoring activities, the LTCA also ensures access to needed cost-effective, high-quality extended and long-term care services for Medicaid beneficiaries residing in home and community-based or institutional settings. The office also provides contract management for various long-term care services and supports contracts, including the long-term care assessment contract, the contracts for the Dual-Eligible Special Needs Plan (D-SNP) and Program for All-Inclusive Care for the Elderly (PACE) programs, and more.

This division contains the following 4 activities**:**

- **Long-Term Care Support Services** – provides administrative support functions to the Long-Term Care Administration;
- **Oversight** – provides quality oversight and assurance, including compliance with mandatory Centers for Medicare and Medicaid Services (CMS) assurances, oversight and audits/site visits, and management of corrective action plans;
- **Operations** – provides day-to-day operations support for LTC partners, providers and beneficiaries to ensure service delivery; training and technical assistance to providers; provider readiness; and grievance and appeals triage and resolution; and
- **Intake and Assessment** – clinical operations unit responsible for assessment and intake processes, coordination with the DC Department on Aging and Community Living (DACL), and other clinical processes within LTCA's purview.

Health Care Policy and Research – maintains the Medicaid and CHIP state plans that govern eligibility, scope of benefits, and reimbursement policies for the District's Medicaid and CHIP programs; develops policy for the Health Care Alliance program and other publicly funded health care programs that are administered or monitored by DHCF based on sound analysis of local and national health care and reimbursement policies and strategies; and ensures coordination and consistency among health care and reimbursement policies developed by the various divisions within DHCF.

This division contains the following 3 activities**:**

- **Health Care Policy** – maintains the Medicaid State Plan, which governs the eligibility, scope of benefits, and reimbursement policies of the Medicaid and CHIP programs;
- **Health Care Policy and Research Support Services** – provides administrative support to functions to the Health Care Policy and Planning Administration; and
- **Eligibility Policy and Oversight** – serves as liaison to District and federal agencies regarding eligibility-related matters; ensures collaboration and coordination between the agencies and facilitates compliance by the Department of Human Services' Economic Security Administration with DHCF eligibility policy; interprets federal and state eligibility rules and regulation; establishes eligibility policies and criteria for the Medicaid and CHIP programs, as well as the Health Care Alliance and the Immigrant Children's Program and determines eligibility for long term care services and supports and special eligibility programs; interprets and helps draft legislative changes, rules and regulations for the District regarding eligibility requirements; manages the Optional State Supplement Payment Program for eligible District of Columbia residents residing in an adult foster care home; and conducts eligibility determinations for Medicaid populations that include Long Term Services and Supports and Special Medicaid Programs.

DC Access System (DCAS) – has responsibility to design, develop, implement, and manage the DC Access System (DCAS), which is an integrated eligibility system for all health and human services for the District. In addition, this administration is responsible for supporting the functionality and funding of all components of DCAS and their seamless interfaces with the Health Benefits Exchange and Department of Human Services program components.

This division contains the following 5 activities**:**

- **DCAS Program Management** – manages all operational and functional activities related to the DCAS project;
- **DCAS Project Management** – manages all project management and functional activities related to the DCAS project;
- **DCAS HHS Functional** – joint oversight development execution of the DC access systems;
- **DCAS Organizational Change Management** – manages all historical, current, and forecasted project initiatives associated with Organization Change Management; and
- **DCAS Information Technology** – manages the operational tasks and maintenance for the DCAS project.

Health Care Operations – ensures the division of programs that pertain to the payment of claims and manages the fiscal agent contract, the administrative contracts, systems, and provider enrollment and requirements. The office provides contract management of the Pharmacy Benefits Manager, the Quality Improvement Organization contract, and the Medicaid Management Information System (MMIS) Fiscal Intermediary contract, as well as additional administrative contracts.

This division contains the following 3 activities**:**

- **Claims Management** – oversees MMIS operations; systems requests; member services;
- **HCOA Support Services** – provides administrative support functions to the Health Care Operations divisions; and
- **Public and Private Provider Services** – manages the Administrative Services Organization contract, provider enrollment and recruitment, and internal and external provider services and inquiries. The office also maintains positive ongoing coordination and continuity with all public provider agencies of the District of Columbia government to enhance each agency's understanding of Medicaid reimbursement policies; is the accountable office within DHCF for implementation of policy that directly impacts other District agencies that serve as Medicaid providers; identifies opportunities to improve the reimbursement procedures of each agency; and works closely with these agencies to review federal policy to ensure that federal reimbursement is being maximized and compliance assured through claims processing and through program development.

Health Care Reform and Innovation – identifies, validates, and disseminates information about new health care models and payment approaches serving Medicaid beneficiaries with the goal of enhancing health care quality, improving care and outcomes, promoting health equity, and enhancing the value and efficiency of DHCF programs. The division creates and tests new delivery systems and payment models among providers in the District and builds collaborative learning networks to facilitate innovation, implement effective practices, and facilitate technology improvements to support delivery system re-design and improvement.

This division contains the following 3 activities:

- **Health Care Reform and Innovative Support Services** – is responsible for advancing the use of information technology among health care providers in the District. These activities include HCRIA's responsibility to design, develop, implement, and sustain Health Information Exchange (HIE) activities. HIE's infrastructure provides the technology, processes, and operations needed to facilitate the exchange of health information between health stakeholders. HCRIA's Health Information Technology (HIT) program offers incentives, outreach, and technical assistance to drive the adoption and use of Certified Electronic Health Records Technology by District health care providers. The program aligns with CMS's Meaningful Use framework.
- **Grant Administration Function** – develops and executes strategies for payment and delivery system reform in the District, including developing, implementing, and monitoring practice transformation activities as well as developing demonstration projects and grants to support various value–based purchasing and practice transformation strategies; and
- **Health Information Exchange** – responsible for advancing the use of digital health among health care providers in the District. This Division serves as the state health IT coordinator and regulates health information exchange (HIE). It is also responsible for the design, development, implementation, and sustainability of DC HIE connectivity and infrastructure, which enables operations needed to facilitate the exchange of health information across the District's health system. Lastly, this Division offers incentives, outreach and technical assistance to drive adoption and meaningful use of digital health tools by District health care providers.

Program Oversight – to provide a reasonable and consistent oversight of the Medicaid program. This division contains the following activity**:**

- **Quality and Health Outcomes** – ensures continuous improvements to the quality and performance principles and practices of the health care programs administered by DHCF.

Provider Services – provides payment to providers in the following: the Medicaid program, the Children's Health Insurance Program (CHIP) and the Alliance program.

This division contains the following 10 activities:

- **1115/1915 - Medicaid** – waiver programs offered through 1915 and 1115 authority providing care for home and community-based services (1915) or demonstration programs(1115) to expand care beyond the traditional Medicaid program;
- **Fee for Services (FFS) - Childless Adults (Group 8)** – fee for Service provider payments for adults between the ages of 21 and 64 who do not have dependents;
- **FFS - CHIP** – fee for service provider payments for the Children's Health Insurance Program. This population is broken out due to a more favorable federal match percentage;
- **FFS Medicaid** – fee-for-service provider payments for the general Medicaid population;
- **MCO Alliance** – MCO payments for Alliance beneficiaries. Health care insurance for District residents who are not otherwise eligible for Medicaid;
- **MCO Childless Adults (Group 8)** – MCO payments for adults between the ages of 21 and 64 who do not have dependents;
- **MCO CHIP** – MCO payments for the Children's Health Insurance Program. Provides coverage for children who are in families with income too high to qualify for Medicaid, but are too low to afford private coverage;
- **MCO Medicaid** – MCO payments for the general Medicaid population;
- **MCO Waiver** – dual Eligible Special Needs Program (DSNP), Medicare Advantage; and
- **Indigent Care** - a fee-for-service program serving Alliance beneficiaries who are District residents not otherwise eligible for Medicaid.

Agency Financial Operations – provides comprehensive and efficient financial management services to, and on behalf of, District agencies so that the financial integrity of the District of Columbia is maintained. This division is standard for all agencies using performance/based budgeting**.**

Agency Management – provides for administrative support and the required tools to achieve operational and programmatic results. This division is standard for all agencies using performance-based budgeting.

The DHCF Agency Management structure also includes:

- **Office of the Senior Deputy Director/Medicaid Director (Office of the State Medicaid Director - OSMD)** – responsible for overseeing the DC Medicaid Program, ensuring its affordability to the District of Columbia residents, including the office of the Chief Medical Officer;
- **Health Care Ombudsman's Office** – responsible for assisting people in understanding their health care coverage and providing guidance, advocating, and assisting in the appeals process;
- **Office of the Senior Deputy Director, Finance (Office of Rates, Reimbursement and Financial Analysis - ORRFA)** – responsible for provider rate setting and reimbursement, program financial analysis of spending trends and utilization and actuarial oversight of the Managed Care Organizations;
- **Data Analytics and Research** – transforms data into meaningful information to support decision-making, effectiveness, and transparency in the District's Medicaid and locally funded health coverage programs; and
- **Program Integrity (PI)** – provide a reasonable and consistent system of oversight of the Medicaid program; reduce and eliminate fraud, waste, and abuse in the Medicaid Program.

Division Structure Change

The Department of Health Care Finance has no division structure changes in the FY 2027 proposed budget.

FY 2026 Approved Budget to FY 2027 Proposed Budget, by Revenue Type

Table HT0-5 itemizes the changes by revenue type between the FY 2026 approved budget and the FY 2027 proposed budget. For a more comprehensive explanation of changes, please see the FY 2027 Proposed Budget Changes section, which follows the table.

Table HT0-5
($ in thousands)

DESCRIPTION	DIVISION/PROGRAM	BUDGET	FTE
LOCAL FUNDS: FY 2026 Approved Budget and FTE		**1,054,707**	**176.0**
Removal of One-Time	Multiple Programs	-15,314	0.0
LOCAL FUNDS: FY 2027 Recurring Budget		**1,039,393**	**176.0**
Increase: To adjust the contractual services budget	Multiple Programs	3,411	0.0
Increase: To align fixed costs with proposed estimates	Multiple Programs	141	0.0
Increase: To align resources with operational spending goals	Multiple Programs	1,127	0.0
Increase: To align personnel services and fringe benefits with projected costs	Multiple Programs	1,034	-4.7
Increase: To support the costs of pre-existing programmatic initiatives	DC ACCESS SYSTEM	20,946	0.0
Increase: To support medical care costs	PROVIDER SERVICES	80,218	0.0
Enhance: To support Cedar Hill Regional Medical Center (one-time)	HEALTH CARE REFORM AND INNOVATION	5,000	0.0
Enhance: To support Medicaid services	PROVIDER SERVICES	5,174	0.0
Enhance: To support Alliance Dental and Vision coverage	PROVIDER SERVICES	3,970	0.0
Enhance: To support Basic Health Program Vision and Dental coverage	AGENCY MANAGEMENT PROGRAM	5,733	0.0
Enhance: To support Alliance coverage	PROVIDER SERVICES	5,043	0.0
Reduce: To realize programmatic cost savings in nonpersonnel services	Multiple Programs	-2,039	0.0
Reduce: To realize savings in personnel services	Multiple Programs	-1,023	0.0
Reduce: To realize savings from February 2026 Alliance reforecast	PROVIDER SERVICES	-27,785	0.0
Reduce: To realize savings from February 2026 Medicaid reforecast	PROVIDER SERVICES	-22,034	0.0
LOCAL FUNDS: FY 2027 Mayor's Proposed Budget		**1,118,310**	**171.2**
DEDICATED TAXES: FY 2026 Approved Budget and FTE		**285,940**	**6.6**

Table HT0-5
($ in thousands)

DESCRIPTION	DIVISION/PROGRAM	BUDGET	FTE
Increase: To align resources with projected revenues	Multiple Programs	182	0.0
Decrease: To realize programmatic cost savings in nonpersonnel services	PROVIDER SERVICES	-59,581	0.0
Enhance: To align resources with projected revenues	PROVIDER SERVICES	9,625	0.0
DEDICATED TAXES: FY 2027 Mayor's Proposed Budget		**236,167**	**6.6**
SPECIAL PURPOSE REVENUE FUNDS: FY 2026 Approved Budget and FTE		**5,492**	**15.1**
Decrease: To align personnel services and fringe benefits with projected costs	Multiple Programs	-21	0.0
Decrease: To align resources with projected revenues	PROVIDER SERVICES	-189	0.0
Decrease: To align resources with projected revenues	Multiple Programs	-501	0.0
SPECIAL PURPOSE REVENUE FUNDS: FY 2027 Mayor's Proposed Budget		**4,781**	**15.1**
FEDERAL GRANT FUND - FPRS: FY 2026 Approved Budget and FTE		**10,400**	**8.0**
Increase: To support additional FTE(s)	Multiple Programs	570	5.0
Decrease: To align the budget with projected grant awards	Multiple Programs	-732	0.0
FEDERAL GRANT FUND - FPRS: FY 2027 Mayor's Proposed Budget		**10,238**	**13.0**
FEDERAL MEDICAID PAYMENTS: FY 2026 Approved Budget and FTE		**4,148,230**	**185.3**
Increase: To align personnel services and fringe benefits with projected costs	Multiple Programs	1,531	4.1
Increase: To reflect the Federal match component of the budget	Multiple Programs	13,455	0.0
Decrease: To reflect the Federal match component of the budget	PROVIDER SERVICES	-115,639	0.0
Reduce: To realize savings in personnel services	Multiple Programs	-586	0.0
Reduce: To adjust for Medicaid reimbursements	Multiple Programs	-80,745	0.0
FEDERAL MEDICAID PAYMENTS: FY 2027 Mayor's Proposed Budget		**3,966,246**	**189.3**
GROSS FOR HT0- DEPARTMENT OF HEALTH CARE FINANCE (HT0)		**5,335,741**	**395.2**

(Change is calculated by whole numbers and numbers may not add up due to rounding)

Note: For more detailed information regarding the proposed funding for interagency projects funded within this agency, please see **Appendix H, FY 2027 Interagency Budgets,** in the **Executive Summary, Volume 1** located on the OCFO's website.

FY 2027 Proposed Operating Budget Changes

Table HT0-6 contains the proposed FY 2027 budget by fund compared to the approved FY 2026 budget.

Table HT0-6

Appropriated Fund	FY 2026 Approved	FY 2027 Proposed	% Change from FY 2026
LOCAL FUND	$1,054,706,805	$1,118,309,748	6.0
DEDICATED TAXES	$285,940,321	$236,166,588	-17.4
SPECIAL PURPOSE REVENUE FUNDS	$5,491,817	$4,780,571	-13.0
FEDERAL GRANT FUND - FPR	$10,399,776	$10,238,139	-1.6
FEDERAL MEDICAID PAYMENTS	$4,148,230,385	$3,966,246,385	-4.4
GROSS FUNDS	**$5,504,769,104**	**$5,335,741,431**	**-3.1**

Mayor's Proposed Budget

Increase: In Local funds, the proposed budget submission for the Department of Health Care Finance (DHCF) includes an increase of $80,218,195 in the Provider Services division to support the increasing cost of medical care for beneficiaries. The Local funds budget proposal also includes an increase of $20,945,932 to support nonpersonnel services costs in the DC Access System (DCAS) division. DCAS is a portal for residents to access health and human services, including Medicaid,

Supplemental Nutrition Assistance Program (SNAP), and Temporary Assistance for Needy Families (TANF). Additionally, proposed Local fund increase of $3,410,988 in contractual services will primarily support operations in the Health Care Operations and Agency Management divisions.

DHCF will allocate additional resources toward the purchase of supplies and equipment, resulting in a Local funds increase of $1,126,578 across multiple divisions. Additional adjustments across multiple divisions to the proposed Local funds budget submission include increases of $1,034,192 in personal services, which comprises a net reduction of 4.7 Full-Time Equivalents (FTEs), to realign salaries and fringe benefits; and $141,170 in nonpersonal services to support projected fixed cost estimates.

In Dedicated Tax funds, the budget proposal includes an increase of $182,172 across multiple divisions, of which $178,680 will align the budget with projected revenues; and $3,492 will align projected salaries and fringe benefit costs for current FTEs.

The proposed budget submission for Federal Grant funds includes an increase of $569,895 to support an additional 5.0 FTEs across multiple divisions.

In Federal Medicaid Payments, a proposed increase of $13,454,741 reflects services provided to District residents that are eligible for reimbursement from Medicaid; and an increase of $1,531,288 and 4.1 FTEs across multiple divisions reflects adjustments to personnel services.

Decrease: The proposed budget submission includes several decreases to the Provider Services division. These adjustments include reductions of $59,581,204 in Dedicated Tax funds, $189,021 in Special Purpose Revenue (SPR) funds, and $115,639,408 in Federal Medicaid Payments. These actions are attributed to projected changes in directed payments and aligning Medicaid matching funds to District spending.

In SPR funds, the budget proposes a net decrease of $21,432 across multiple divisions will align salaries and fringe benefit costs; and a net decrease of $500,793 across multiple divisions will align with projected revenues, primarily from the Medicaid Collections-3rd Party Liability fund.

The proposed budget submission for Federal Grant funds reflects a decrease of $731,533 across multiple divisions to align with projected grant awards.

Enhance: The proposed Local funds budget includes several enhancements to support the agency's mission of improving the health outcomes for District residents. These adjustments include an increase of $5,733,009 in the Agency Management division to support Basic Health Program (BHP) dental and vision benefits. Additionally, an overall increase of $14,186,880 is proposed in the Provider Services division, of which $5,173,619 will cover rising medical care costs; $5,043,229 will support Alliance coverage; and $3,970,032 will support Alliance dental and vision benefits. Lastly in Local funds, a proposed one-time increase of $5,000,000 in the Health Care Reform and Innovation division will support the Cedar Hill Regional Medical Center.

In Dedicated Tax funds, the proposed budget includes an increase of $9,625,299 in subsidies in the Provider Services division to align resources with the projected revenues.

Reduce: DHCF's Local funds budget proposal includes several reductions across multiple divisions. These adjustments include decreases of $1,022,820 in personnel services across multiple divisions to realize savings from vacant positions; $2,038,750 in nonpersonnel service cost across multiple divisions, of which $1,213,250 reflects savings in contractual services and $825,500 reflects savings in equipment costs. Lastly in Local funds, the budget proposal includes an overall reduction of $49,818,896 in subsidies in the Provider Services division due to the February 2026 reforecast, which reflects savings of $22,034,386 from Medicaid and $27,784,510 from the Alliance.

In Federal Medicaid Payments, the proposed budget includes a decrease of $585,529 in personnel services across multiple divisions, to realize savings from vacant positions. Additionally, a proposed decrease of $80,745,091 in subsidies reflects adjustments for Medicaid reimbursable services provided, primarily in the Provider Services division.

FY 2027 Proposed Full-Time Equivalents (FTEs)

Table HT0-7 contains a summary of the FY 2027 proposed budgeted Full-Time Equivalents (FTEs).

Table HT0-7

Total FY 2027 Proposed Budgeted FTEs	**395.2**
Less: Interagency FTEs budgeted in this agency but employed by other agencies:	
BY0-DEPARTMENT OF AGING AND COMMUNITY LIVING	(5.0)
CB0-OFFICE OF THE ATTORNEY GENERAL FOR THE DISTRICT OF	(0.9)
FL0-DEPARTMENT OF CORRECTIONS	(1.0)
HC0-DEPARTMENT OF HEALTH	(3.6)
HG0-OFFICE OF THE DEPUTY MAYOR FOR HEALTH AND HUMAN SE	(1.0)
JZ0-DEPARTMENT OF YOUTH REHABILITATION SERVICES	(1.0)
PO0-OFFICE OF CONTRACTING AND PROCUREMENT	(13.0)
TO0-OFFICE OF THE CHIEF TECHNOLOGY OFFICER	(1.2)
Total Interagency FTEs budgeted in this agency, employed by other agencies	**(26.7)**
Add: Interagency FTEs budgeted in other agencies but employed by this agency:	
JA0-DEPARTMENT OF HUMAN SERVICES	2.6
Total Interagency FTEs budgeted in other agencies, employed by this agency	**2.6**
Total FTEs employed by this agency	**371.0**

Note: Table HT0-7 displays the impact of the buyer agencies budgets funding the seller agencies FTEs in the FY 2027 budget, compared to how FTEs were budgeted in FY 2026.

-It starts with the FY 2027 budgeted FTE figure, 395.2 FTEs.

-It subtracts 26.7 FTEs budgeted in HT0 in FY 2027 who are employed by another agency.

-It adds 2.6 FTEs budgeted in other agencies in FY 2027 who are employed by HT0.

-It ends with 371.0 FTEs, the number of FTEs employed by HT0, which is the FTE figure comparable to the FY 2026 budget.